

THE POST-SURGERY REBUILD ROADMAP

Four priorities for the body you have now

JT Boone | Ostomy • J-Pouch • Reversal

Surviving surgery and rebuilding afterward are two different jobs. If your day still revolves around your abdomen, bowel function, food decisions, weakness, or fear of a setback, start by organizing the whole picture.

This is a reflection guide, not a universal exercise prescription. Keep your surgical and GI team's current restrictions at the center of your plan.

1. ASSESS

Know your starting point

Write down your current anatomy, surgeries and dates, active concerns, and any lifting or exercise restrictions. Choose three specific activities you want back: getting out of bed, carrying groceries, walking, traveling, or returning to training.

For each activity, describe what happens now and rate your ability from 0 (unable) to 10 (your usual ability). Use the same activity and rating when you check again.

Ask: What is difficult, uncomfortable, or something I am afraid to do because of my abdomen or surgeries?

2. STABILIZE

Create enough capacity to practice

Look at food and protein you tolerate, your prescribed hydration and electrolyte plan, bowel or stoma output, energy, sleep, and stress. Bring changes or unresolved concerns to the team managing your care.

Choose one practical recovery habit to work on first. A movement plan is easier to follow when your daily capacity supports it.

Ask: What leaves me more stable, and what repeatedly leaves me depleted?

3. RESTORE

Rebuild how breathing and movement work together

Corrective abdominal work starts with how you breathe, coordinate your trunk, and move during a task. Depending on your assessment and restrictions, your practice might include supported breathing, gentle abdominal coordination, or a controlled transfer or walking task.

The starting point is individual. A harder exercise is not automatically a better exercise. Practice the movement you can control, then review your response during the task and later that day or the next day.

For some people, in-person work for scar sensitivity, abdominal tightness, and restricted movement may be a useful part of the plan. Its suitability depends on healing, symptoms, consent, and the practitioner's training. Remote coaching can help organize questions and local referrals.

Ask: Can I do this with comfortable breathing and control, and how do I feel afterward?

4. REBUILD

Make progress visible in real life

Progress from a manageable starting point toward the activities you chose. Change one part of the exercise dose at a time and review your response before increasing it again.

Track more than exercise difficulty: the same three activities, confidence, symptoms, and daily quality of life. If your setup or restrictions change, record that context so comparisons remain meaningful.

Ask: What can I do more comfortably, confidently, or independently now?

YOUR FIRST PAGE

My current restrictions or questions for my team: _____

The three activities I want back: _____

The one priority I will start with: _____

What I will track and check again: _____

WHEN YOU WANT A PERSONALIZED PLAN

The \$1,000 30-Day Post-Surgery Rebuild Intensive includes four private sessions, a Personal Rebuild Map, an individualized corrective routine, baseline/midpoint/final checks, and reasonable between-session support.

The movement framework draws on my CHEK training: assess, develop coordination and control, then progress capacity. Your history and response determine your plan.

Start with “See If I Can Help.” Tell me what you have been through, what activities are difficult, and whether you have current restrictions or an active complication. I review your answers before discussing enrollment. A fit call is optional.

See If I Can Help: [See If I Can Help](#)

Already invited to talk? [Book your invited fit call](#)

jtboone.com

GET MEDICAL HELP WHEN NEEDED

New or worsening severe abdominal pain, vomiting, marked swelling, or inability to pass stool/gas can require urgent medical evaluation. Do not exercise through a suspected obstruction or active complication. Coaching and bodywork complement your medical care.

Educational sources: CHEK Core Conditioning; Program Design; Advanced Program Design; Scientific Back Training; How to Eat, Move and Be Healthy! Medical context: NIDDK, Abdominal Adhesions; NHS, Ileostomy Recovery.

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